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Multi Agency Information Sharing (MAIS) Standard Development Working Group

3rd September 2025

PRIVATE AND CONFIDENTIAL

Agenda

1. Welcome (5 mins)
2. Programme Recap (10 mins)
3. Adult Social Care (5 mins)
4. Person Specification (20 mins)
    a. Gender
    b. Date of Birth
    c. Moving forward
5. Data Exchange Standard (25 mins)
6. Taxonomy for MAIS (20 mins)
7. Next Steps (5 mins)

Things to be aware of

Conflicts of interest – Please inform the chair / secretariat if you think there is a risk.

Anti-competitive activities – Please inform the chair / secretariat if you think there is a risk of anti-competitive activity.

Intellectual Property - If you are concerned about the use of your contributions, please contact the secretariat.

Meeting Recording - We will record the meeting and using AI transcription tools.

Working together

1. Every voice matters
2. Please join in on the conversation, we don’t want this to be one way
3. If you’re happy to, keep your camera on
4. Please raise your hand if you want to come in, or drop your ideas in the chat
5. Respect the diverse viewpoints in the room, assume positive intent and remain open for different experiences
6. We will use Mentimeter for audience interaction

Thanks for your interactions on GitHub!

Open 22 Closed 11 Author

[Content]: Should the person spec include a photo of the person
Feature #41 · kyongCoram opened 3 weeks ago

[Presentation]: Future proofing / backward compatability To discuss
#37 · andjnewman opened on Jul 17

Issue regarding People Spec Scope documentation To discuss
#36 · andjnewman opened on Jul 17

[Content]: National Insurance Number Attributes To discuss
#33 · andjnewman opened on Jul 17

Issue regarding People Spec Specification - DoBs Attributes To discuss
#31 · stephencairns opened on Jul 10

Issue regarding People Spec Specification - addresses aren't personal
#30 · AlistairHerbert opened on Jul 10

Data standards cross-over with D2I SSD Other Standards
#25 · SLornieCYC opened on Jul 7

Issue Regarding People Spec Specification: Gender vs Sex Attributes
#24 · SLornieCYC opened on Jul 7

Issue Regarding People Spec Context - child to adult transition
#21 · juliehaw opened on Jul 7

Unique Identifiers for Unborn Children

Issue Regarding People Spec Specification: Gender vs Sex #24

Open

SLornieCYC opened on Jul 7 · edited by SLornieCYC

Currently the draft specification includes Gender and not Sex ("The person's state
biological sex.").

This is inconsistent with the data that DFE expect LAs to provide through statuto
Gender). It is also inconsistent with how our existing children's CMS (Mosaic) con
Sex and Gender but uses Sex as part of the client matching to the NHS).

Data item required in the children looked after return and child in need census
https://www.gov.uk/government/publications/children-looked-after-return-2024-
https://www.gov.uk/government/publications/children-in-need-census-2024-to-2
https://www.gov.uk/government/publications/common-basic-data-set-cbds-data

> Children looked after should have their sex reported, as recorded on a birth ce
> This is a value which identifies the sex of a person as recognised in law.

> Children in Need should have their sex reported, as recorded on a birth certific
> a value which identifies the sex of a person as recognized in law.

Create sub-issue

howaskew on Jul 8

Thanks for this detail. Here's @NevadaM's notes preparing the first draft of the sp

Gender and Sex

The way that the gender and sex of a person is represented in their data can lead to a number of issues in health and care settings if not carried out with appropriate standards in place. We noted that this is a topic of importance across the public sector, and we therefore sought to fully evaluate our options.

Regarding phenotypic sex, we first saw that few of the data standards we examined had fields for the information. While MODS, CIS, and CBDS each contained references to phenotypic sex, the others simply did not, including the two most clinical standards FHIR and the Healthy Child Record Standard. In both, the field is deemed "unnecessary", and we met a consensus that for our use cases, it would be unnecessary too.

Gender fields, in contrast, were present in all our analysed data standards, presumably because of its importance not only as a search

ALPHA - This website is currently under development. Please provide feedback on Github

Data Standards for Social Care

Use Cases Standards Working Groups Specifications

This catalogue provides access to the data specifications being developed for social care case management systems.

The specifications are generated dynamically from the latest versions in GitHub.

As new specifications are developed they will be added to this catalogue.

Title	Status	Link	Issues
Person Data Standard (Identification)	Draft	Person Specification	Issues
Placements Data Standard	Draft	Placements Specification	Issues
Data Exchange Standard	Draft	Data Exchange Specification	Issues

https://socialcaredata.github.io/spec/

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Mentimeter

What type of organisation are you from?

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Local Gov	Central Gov	System vendor	Prof. Body	Consulting/Advisory	Other
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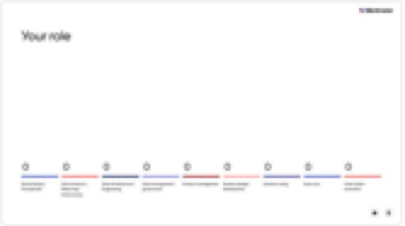
Standards Working Gro...

Choose a slide to present



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Your role

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Social Worker / Principle SW	Data Analytics / Reporting / Performance	Data Architechture / Engineering	Data management / governance	Product management	Systems design / development	Systems config	Executive	Other (add a comment)
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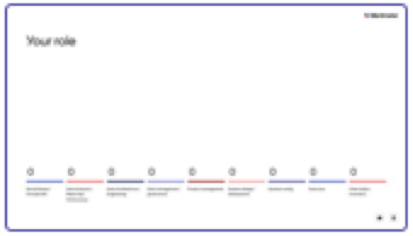
Standards Working Gro...

Choose a slide to present

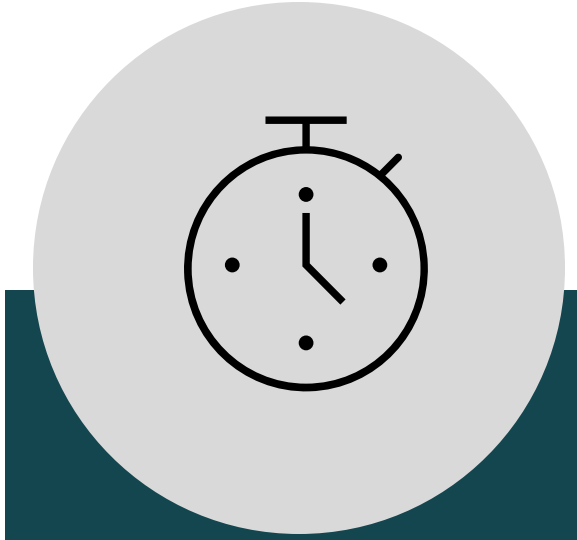


Programme Recap

We're interested in using standards to drive four key benefits:

Improved data sharing to support frontline decision making

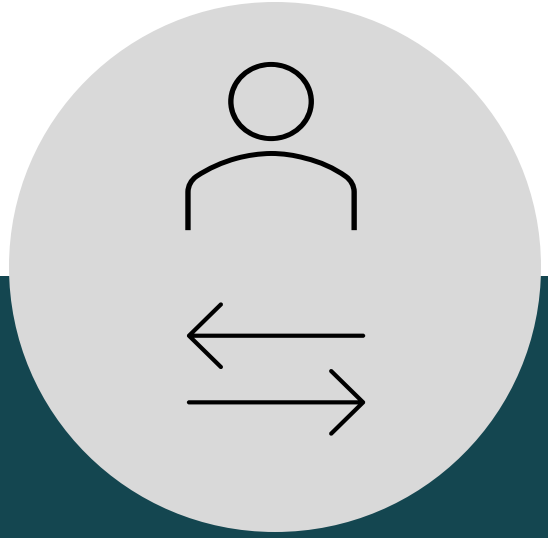
Better analysis to support strategic decision making



Access to productivity - enhancing tools for frontline staff

Enabling those using social care to see and contribute to their record

Our initial focus



MAIS

- Person standard
- Data Exchange standard
- Taxonomy

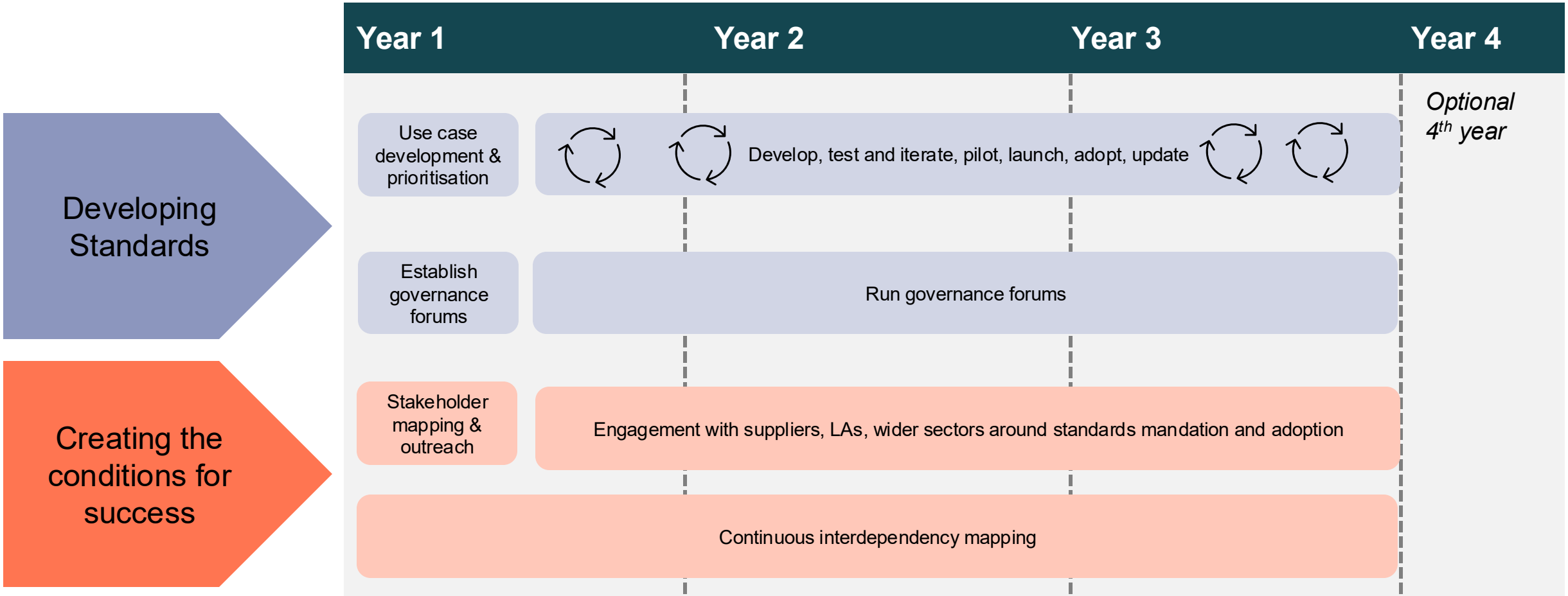
2nd Working Group session (TODAY)

Placements

- Standard for describing placements
- Piloting in the SW region

2nd Working Group session on the 17th September

What might the next 3 years look like to help us solve the problems in the market using standards?



Adult Social Care

Adult Social Care Data Standards

- Adult social care and children's social care face many of the **same challenges** with the **same systems**

- Developing standards across social care should maximise the benefit of the programme

This programme is now a joint commitment from DfE and DHSC

ASC will integrate at use case level

Use Cases	In development	Future development
CSC only	Placement	To be prioritised
ASC only		To be prioritised
Both	**MAIS**	To be prioritised

Standards for MAIS

Our Standards for Multi-Agency Information Sharing (MAIS)

Person Standard:
Defining which fields should be standardised for a person

2nd draft for review (by the end of session)

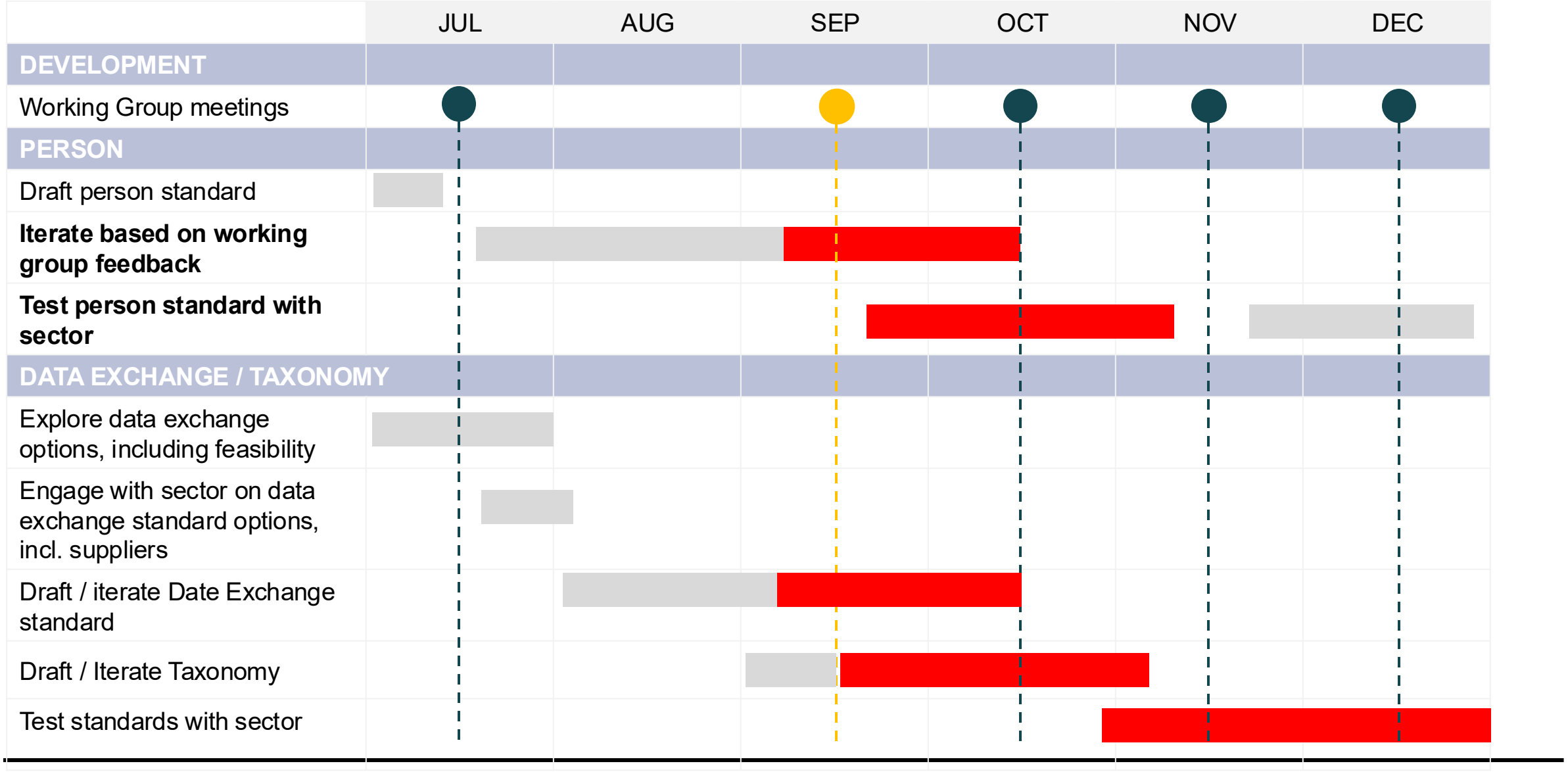
Data Exchange Standard:
To support movement of data between systems

1st draft for comments

Taxonomy:
To define a shared language to support data exchange

1st draft for comments

Next Steps for MAIS



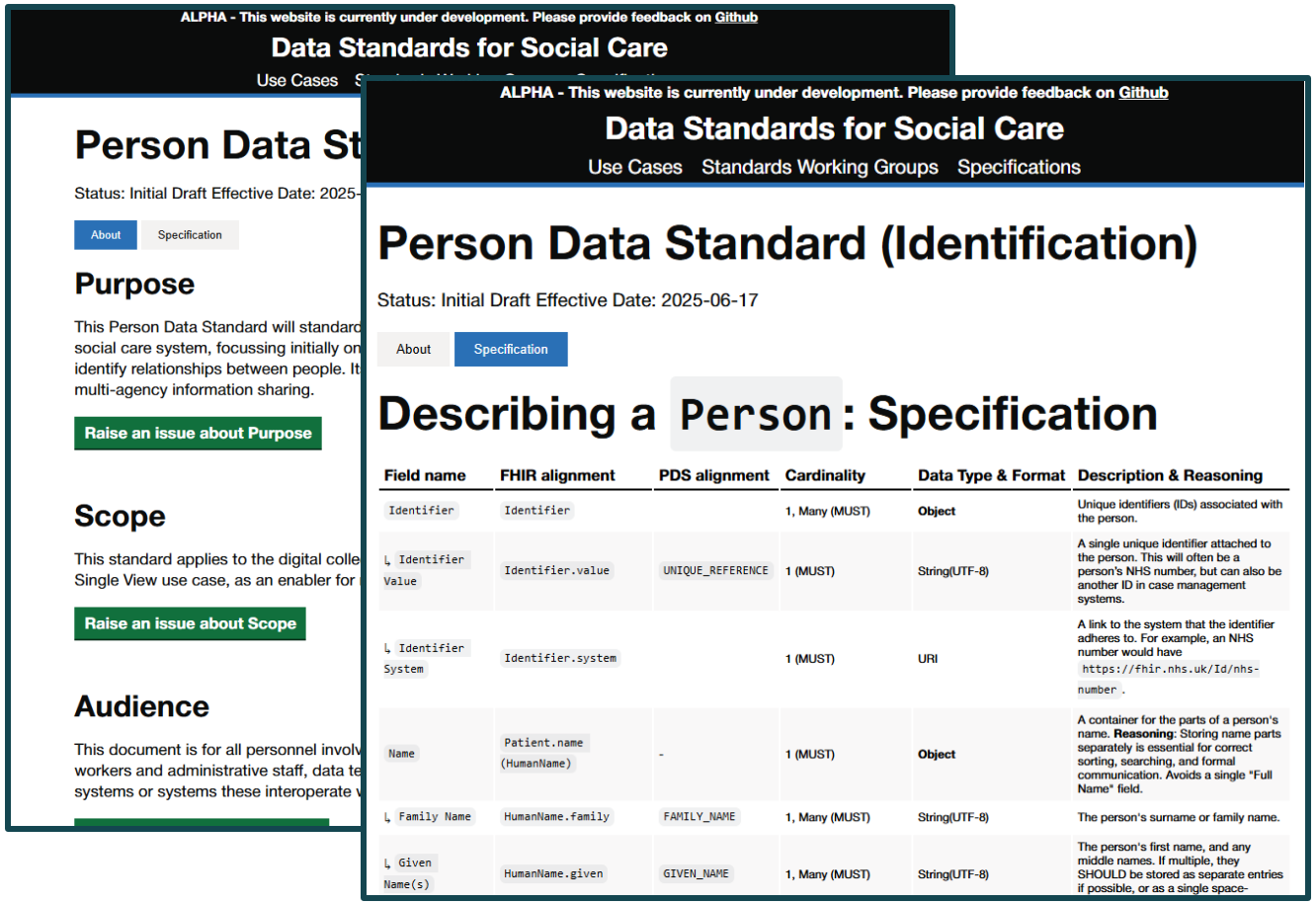
Q&A – Overall Programme

Person Standard

Our Standards for MAIS

Person Standard: Defining which fields should be standardised for a person

2nd draft for review



ALPHA - This website is currently under development. Please provide feedback on Github

Data Standards for Social Care

Use Cases Standards Working Groups Specifications

Person Data Standard (Identification)

Status: Initial Draft Effective Date: 2025-06-17

About | Specification

Describing a `Person`: Specification

Field name	FHIR alignment	PDS alignment	Cardinality	Data Type & Format	Description & Reasoning
`Identifier`	`Identifier`		1, Many (MUST)	**Object**	Unique identifiers (IDs) associated with the person.
↳ `Identifier Value`	`Identifier.value`	`UNIQUE_REFERENCE`	1 (MUST)	String(UTF-8)	A single unique identifier attached to the person. This will often be a person's NHS number, but can also be another ID in case management systems.
↳ `Identifier System`	`Identifier.system`		1 (MUST)	URI	A link to the system that the identifier adheres to. For example, an NHS number would have `https://fhir.nhs.uk/Id/nhs-number`.
`Name`	`Patient.name (HumanName)`	-	1 (MUST)	**Object**	A container for the parts of a person's name. **Reasoning**: Storing name parts separately is essential for correct sorting, searching, and formal communication. Avoids a single "Full Name" field.
↳ `Family Name`	`HumanName.family`	`FAMILY_NAME`	1, Many (MUST)	String(UTF-8)	The person's surname or family name.
↳ `Given Name(s)`	`HumanName.given`	`GIVEN_NAME`	1, Many (MUST)	String(UTF-8)	The person's first name, and any middle names. If multiple, they SHOULD be stored as separate entries if possible, or as a single space-

Our goal: A standard for the minimum amount of data necessary to *identify* a person in social care

Updates on the Person Standard

Since the last meeting we:

- Updated the spec to accept multiple different identifiers beyond the NHS number
  - (#23 & #34) Not everyone has an NHS number, and we need to facilitate data records where persons are identified by one or more non-NHS identifiers.

- Decided to continue to include address in the person spec due to its utility in "searching" for a person on a system.
  - (#30) Given the usage of address information for the "searching" for a person on a system, we have decided to not do this now, we may revisit this decision in the future.

- Made some small changes to the wording of the spec in response to several issues.
- Made some tweaks to the website.

Regular email updates

GitHub Issues Pages

Recommendation 1: Gender and Sex in the Person Standard Issue #24

In our draft: we have an attribute for person-stated gender, not their phenotypic sex.

The issue: This is inconsistent with the data that DfE expect LAs to provide through statutory returns (clarified this year to be Sex and not Gender). It is also inconsistent with how existing children's CMSes connect to the NHS via CPIS. (eg. Mosaic can record both Sex and Gender but uses Sex as part of the client matching to the NHS).

On the one hand...

- Clinical data standards like FHIR or the Healthy Child Record Standard do not use phenotypic sex to describe a person, deeming it "unnecessary"
- In the sensitive context of social care, references to phenotypic sex can raise a number of issues, even when it comes to basic record-keeping

On the other...

- The focus on phenotypic sex, at a broader level, is more than DfE guidance, it's almost a core policy choice of the government
- Some data standards, like ASC MODS, CIS, and CBDS, contain Sex – all of which being developed or in-use in the context of UK social care

OUR RECOMMENDATION

- Keep Gender as a mandatory field for a person, using the same code
- Add a field for Phenotypic Sex which is optional

Do you agree with this proposal? Will it create confusion? Is it too far removed from government advice on the subject? Are we too cautious? Should we stick to our first iteration instead?

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Mentimeter

Gender / Sex

What was the reason for your answer?

All responses to your question will be shown here

Each response can be up to 200 characters long

Turn on voting to let participants vote for their favorites

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Menti

Standards Working Gro...

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Recommendation 2: Date of Birth in the Person Standard Issue #31

In our draft: our attribute for DoB does not facilitate estimated, expected, or unknown DoBs.

The issue: Estimated DoBs are commonly recorded in the context of social care, as are expected DoBs. Strictness around dates can also present an obstacle to searches for people in the social care system.

Solution I

The UK implementation of FHIR allows DoB to be estimated. The data is generally still an ISO 8601 date, but can sometimes be truncated.

```
patient.birthDate: 2002-04-25
```

In this case, the DoB may be estimated, or may be definite.

```
patient.birthDate: 2002
```

In this case, the DoB is estimated.

Solution II

An in-development version of the Australian implementation of FHIR turns DoB into an object with two data points: a date, and an accuracy indicator.

```
Patient.birthDate.value: 2002-04-25
Patient.birthDate.accuracy: UEA
```

Here, UEA means {Unknown-Estimated-Accurate}. The year is unknown, the month is estimated, and the date is accurate.

OUR RECOMMENDATION

- Adopt Solution II

Solution II represents a machine-readable way to convey whether DoB is uncertain, and to what extent it is.

What are your requirements for an accuracy indicator? Do you need more information than a triplet code? Do you think this is helpful, or more hassle?

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Reload

Future versions of the person specification...

The following issues are on our radar, but we do not expect to address them imminently:

- How the standard deals with the transition from a child to adult.
- Should our specification include a photo?
- What is current practice around issuing and allocating identifiers to individuals?
- Exploring best practice on using national insurance number as an identifier.
- Exploring overlaps with data2insight's safeguarding dataset SSD
- Exploring the nuances of addressing: Types of address, confidence in address, unknown addresses, sensitivity of addresses.

Are any of these issues critical?

Are there any other issues we should be focusing on in the near future?

Q&A on Person Spec

Data Exchange Standard

Data Exchange Standard – Context and Roadmap

Core challenge for many front line use cases:

- Care coordination
- Hospital discharge
- Front door risk assessment

Components of the challenge:

- Discover other systems
- Send/receive message between systems
- Establish which people (or places) we are talking about
- Apply rules governing whether or not info can be shared

Principles:

- Accept this requires new national infrastructure
- Start with very small use case – infrastructure challenges are already significant and we don't want to add unnecessary complexity
- Build on this subsequently, probably separate ASC and CSC extensions

Data Exchange Standard – Context and Roadmap

Roadmap

Initial objective is to enable most basic form of data exchange between systems to unlock single view capabilities for all Local Authorities.

Our data exchange standard, person standard and taxonomy are the minimum versions required to achieve this. However, they are developed with longer term goals in mind.

This includes:

- Enabling exchange between existing centralised single view systems.
- Facilitating automated data exchange where appropriate information governance thresholds are met.
- Expanding the data model to facilitate more complex data linking to provide a richer context for a person to understand risk and assess needs.

Data Exchange Standard – Design Decisions

Guided by the programme's standards development principles, we are progressing on the basis of the following assumptions and decisions:

FHIR (Fast Healthcare Interoperability Resources – pronounced 'fire') is a global, open standard for healthcare data exchange.

> *Given the extensive adoption in health and the NHS and familiarity to system suppliers, we are aligning with FHIR standards where possible.*

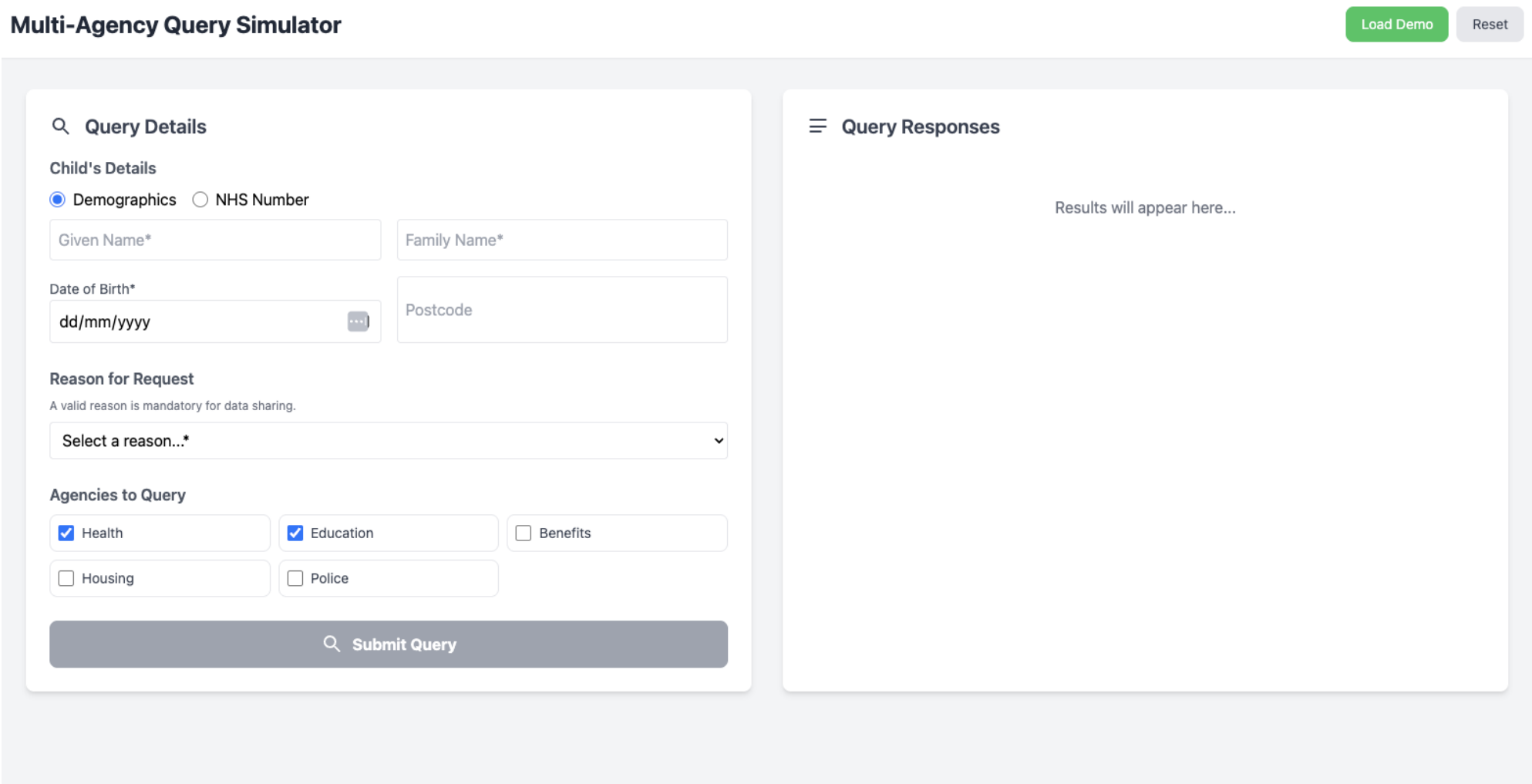
FHIR offers two primary paradigms for data exchange: RESTful API and Messaging.

RESTful APIs follow the principles of Representational State Transfer (REST) over HTTP. This is the most common and often the default approach when discussing FHIR integration.

Messaging is designed for event-driven, asynchronous communication, where a specific event in one system triggers the sending of a collection of FHIR resources to another system.

> *Considering a range of factors including burden of implementation and data minimisation, we are proposing a REST API model*.

Data Exchange Standard – Example exchange via API



Data Exchange Standard – Matching

- The implementation of the matching operation may vary between systems.
- We need to ensure that organisations making requests can trust the information contained in responses.
- Organisations (systems) will be expected to meet a set standard in data matching.
- We expect to create a reference implementation containing synthetic data to emulate common and challenging match conditions.
- Another area to explore is how to report close or near miss matches.
  For example, should the API response allow for Yes / No / Maybe?

If you have insights and ideas to contribute here, please:

- **share ideas on GitHub**
- **join us for a deep dive workshop (details to follow)**

Data Exchange Standard – Information Governance

Problem Statement: To achieve broader Multi-Agency Information Sharing through secure system to system data exchange, we need to be able to reference the appropriate authority permitting data sharing.

High level authority:

Working Together
To Safeguard Children
2023

Individual / Local Agreements:

e.g. Somerset Safeguarding
Children Partnership
Information Sharing Agreement

Manual interpretation of Data Processing Agreements (DPAs) and use cases in social care data sharing can lead to delays, inconsistency, and increased compliance risk.

A codified, machine-readable framework is needed to automate request validation and compliance.

If you have insights and ideas to contribute here, please:

- **share ideas on GitHub**
- **join us for a deep dive workshop (details to follow)**

Q&A on Data Exchange

Next Steps

- We will be running deep dive workshops on the following:
    - Matching – current practice and matching thresholds?
    - Information Governance – how to enable compliant data sharing through codified agreements

Taxonomy for MAIS

Taxonomy for MAIS

To improve Multi-Agency Information Sharing, we need to unlock the data held in different partner systems.

This means standardised machine to machine, or system to system, data exchange.

For systems to talk to each other, they need a shared language.

We will develop set of data resources to organise and describe key concepts and elements in Social Care to provide that shared language.

Initial resources take the form of controlled vocabularies, code lists and registers.

Two main benefits:

- Enables efficient and consistent communication between systems
- By agreeing on these key vocabularies and agreeing on the rules governing their use, we can start to build trust in more automated data exchange.

Taxonomy for MAIS – Initial Scope

The initial scope is limited to those concepts needed to support the Multi Agency Data Sharing (MAIS) use case and promote the use of single view systems.

For example, we want systems to be able to consistently and coherently make and respond to requests such as:

"This is a request from organisation 1, to organisation 2, for information about any service involvements and related professionals for this person, and it is made under the aegis of use case 1 within DPA1"

Hence our focus so far is on:

- Organisations
- Service Involvements
- Information Governance and Data Processing Agreements

Organisations

We need to catalog al ests to.

Working Together To S

Chapter 4: Organisational responsibilities 110

People in positions of trust 112

Individual organisational responsibilities 113

Schools, colleges and other educational providers 113

Early years and childcare 114

Health 114

Police 117

Adult social care services 119

Housing services 119

Homelessness duty 120

British Transport Police 121

The Prison Service 121

Mother and baby units in prisons 124

The Probation Service 124

Multi-agency public protection arrangements 126

Serious violence duty 128

Children's homes 128

The secure estate for children 129

Youth offending teams 130

UK Visas and Immigration, Immigration Enforcement and the Border Force 130

Children and Family Court Advisory and Support Service 130

The armed services 131

Channel panels 131

Voluntary, charity, social enterprise, faith-based organisations, and private sectors 132

Sports clubs/organisations 133

Organisations

Previous work identified the following as key:

Category	Organisations
Health	GP (Registered surgery)
	CAMHS / Healthy Young Minds
	Community Mental Health team
	Community Nurse / Midwife / Health Visitor
Justice	Probation
	Youth Offending Services
Police	
Tax	Council Tax (Addresses)
Education	School
	School Nurse
Housing	Housing associations
	Temporary housing agencies

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Organisation – Register / Catalogue

Likely output of this work is a register of organisations, built from existing canonical sources.
It will contain the following information for each:

- Identifier
- Name of the organisation
- Type of organisation (not critical, but useful for data collection and verification)
- Status – Is the organisation able to make and respond to requests?
- Verification – Is the organisation's system sending and/or responding requests to the agreed standard?
- Information Governance – references to relevant data processing agreements.

Discussion:

What other information do we need to record about organisations to build trust in data sharing and enable single view functionality?

Service Involvements

We need to be able to refer to service involvements, which requires an ability to classify services. Within service involvements, we need to be able describe professionals / practitioners and their roles.

Our early draft of this contains the following details:

- Identifier
- Name of the service
- Type of service
- Type of Involvement
- Start and end dates of service involvement
- Status
- Practioners
- Contact Details

Discussion:

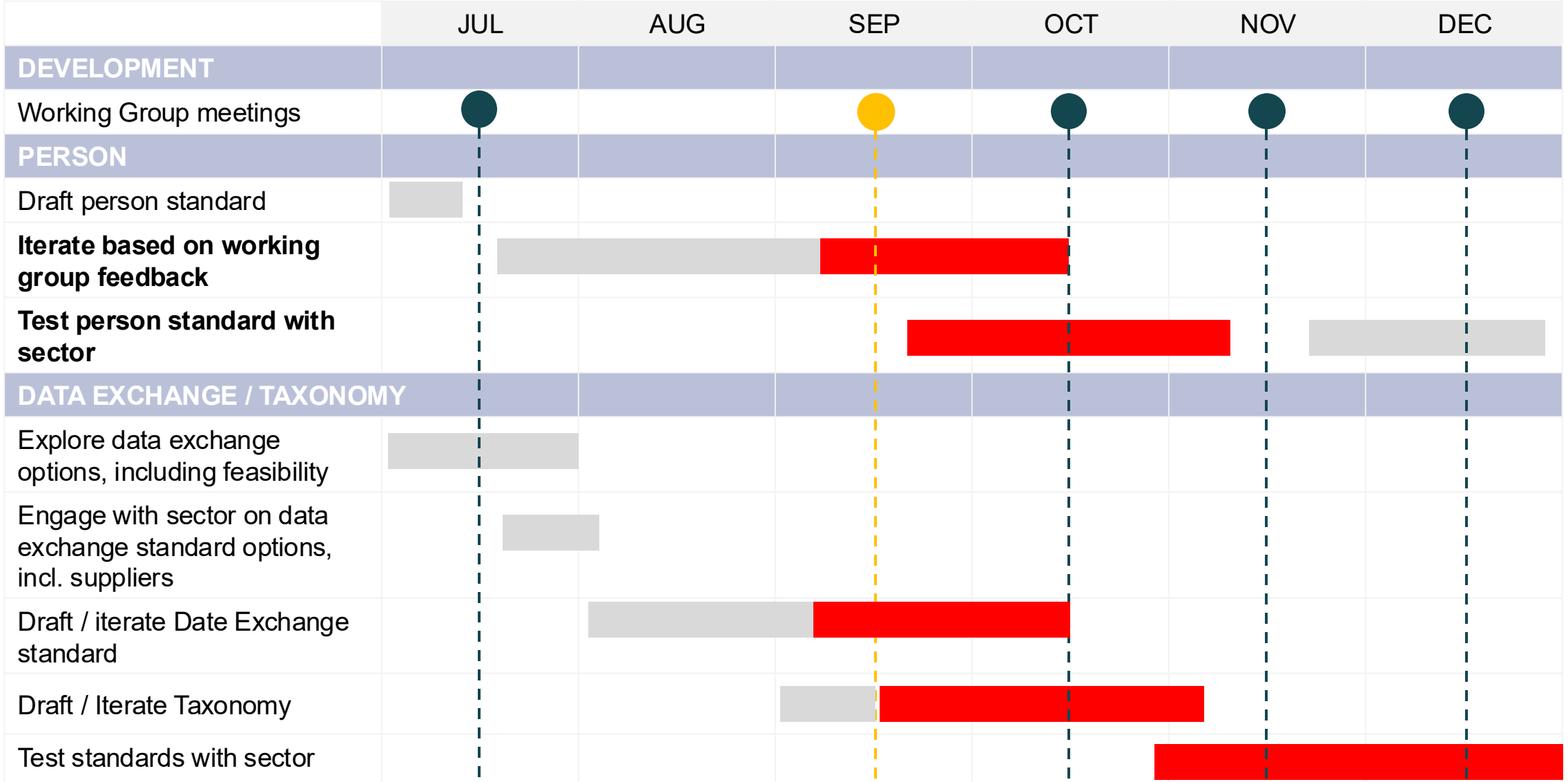
1. **What level of classification of 'services' is needed here?**

2. **How do people define service involvements currently?**

Q&A on Taxonomy

What's next?

Next Steps for MAIS



Find us on GitHub

Website: https://socialcaredata.github.io/ (start here!)

GitHub: https://github.com/SocialCareData

We are using GitHub to:

- Store our specifications
- Track feedback on specifications (Issues)
- Openly share project documentation
- Have Discussions to inform the development of standards

- In the future we will use GitHub to:
- Host and manage the development of any infrastructure or tools we need to develop
- Enable people to reuse our specifications and code

We will work to keep this simple and provide guidance when needed.

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Thank you.

Contact:

MAIS:

Andrew.newman@theodi.org